

DISCHARGE SUMMARY

PATIENT INFORMATION

Name: Patient 05

Age: 62 years old | Sex: Male

MRN: 000-TEST-05

Admission Date: [MM/DD/2026] | Discharge Date: [MM/DD/2026]

Length of Stay: 4 days

PRIMARY DIAGNOSIS

Congestive Heart Failure with reduced ejection fraction (HFrEF), EF 28%

SECONDARY DIAGNOSES

- COPD (GOLD Stage II, on home O2 therapy)
- Peripheral artery disease with claudication
- Hypertension
- Type 2 Diabetes Mellitus
- Hyperlipidemia

HOSPITAL COURSE

Patient 05 presented via ED with dyspnea on exertion, orthopnea x 3 days, and lower extremity edema. Initial vitals notable for HR 104, BP 152/88, RR 22, SpO2 88% on RA. CXR demonstrated bilateral pulmonary edema. Labs significant for BNP 1247 pg/mL, Cr 1.4 mg/dL, Na 134 mEq/L. Echocardiogram confirmed EF of 28% with global hypokinesis. Patient was initiated on IV furosemide protocol with target net negative 2–3 L daily. Underwent diuresis with improvement in symptoms and respiratory status (SpO2 improved to 94% on 2L NC by discharge). Electrolytes repleted PRN. No acute coronary syndrome on troponin serial x 3. COPD exacerbation managed with albuterol/ipratropium nebulizer treatments q4h and methylprednisolone taper. Lower extremity duplex completed—no acute DVT. Physical therapy cleared patient for ambulation with assistive device due to bilateral claudication limiting exercise capacity.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Duration
Furosemide	80 mg	PO	BID	Ongoing; titrate PRN
Lisinopril (ACEI)	20 mg	PO	q.d.	Ongoing
Carvedilol (beta-blocker)	25 mg	PO	BID	Ongoing
Spironolactone (K-sparing diuretic)	25 mg	PO	q.d.	Ongoing
Atorvastatin	40 mg	PO	q.d.	Ongoing
Metoprolol tartrate	50 mg	PO	BID	Ongoing
Albuterol/Ipratropium	2.5/0.5 mg	Nebulized	q6h	Ongoing
Prednisone	20 mg taper	PO	q.d. x 7d then DC	7 days
Aspirin	81 mg	PO	q.d.	Ongoing

FLUID AND DIETARY MANAGEMENT

- Fluid restriction: 1.5 L daily (strict adherence critical given EF 28%)
- Low-sodium diet (<2g NaCl/day)
- Diabetic, heart-healthy diet consult provided

ACTIVITY STATUS

- No strenuous activity or heavy lifting (>10 lbs) × 6 weeks
- Ambulation with cane as tolerated; avoid prolonged standing >15 min due to claudication
- Encourage light walking 10–15 min daily as tolerated

DAILY MONITORING

- Daily weights at same time each morning (report gain >2–3 lbs to clinic)
- Strict I&O log
- Vital signs BID, especially BP given